

Pembrolizumab + pemetrexed + cisplatin

Lung · NCCP 00569 · version 5

Your medicines

Your treatment schedule

A picture of side effects

Numbers connect each symptom box to the relevant anatomical area. Tell your team about new or worsening symptoms.

Pembrolizumab · immune effects

Pemetrexed / cisplatin · chemotherapy

6 Hearing / nerves

Cisplatin

Report tinnitus, hearing change, numbness or tingling before it worsens.

3 Heart

Pembrolizumab

Chest pain, palpitations, fainting or new breathlessness needs urgent assessment.

5 Kidneys / hydration

Pemetrexed / cisplatin / pembrolizumab

Kidney function is monitored closely. Cisplatin needs hydration; immune nephritis can also occur.

7 Blood / infection

Pemetrexed / cisplatin

Fever, shivers, bruising or sudden illness can reflect low blood counts and needs prompt contact.

2 Lungs

Pembrolizumab

New cough or breathlessness needs prompt assessment for pneumonitis and other causes.

4 Liver / bowel

Pembrolizumab

Yellow eyes, dark urine, persistent diarrhoea or abdominal pain can signal immune inflammation.

6 Hearing / nerves

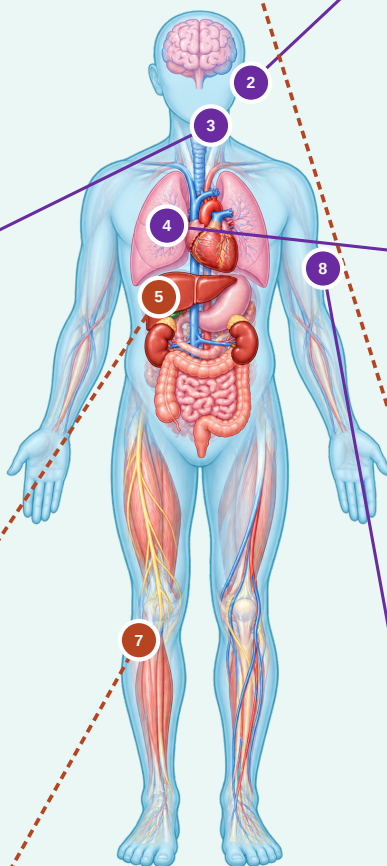
Cisplatin

Report tinnitus, hearing change, numbness or tingling before it worsens.

8 Skin / mouth

Pembrolizumab / pemetrexed

Report severe or blistering rash and mouth soreness affecting fluids.



Selected effects only: the same symptom can have more than one cause, and not every possible side effect is shown.

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TREATMENT JOURNEY

Cycles 1-4 · day 1	Cisplatin phase	Maintenance	Throughout
Pembrolizumab 200 mg + pemetrexed 500 mg/m ² + cisplatin 75 mg/m ² every 21 days.	Cisplatin is given in cycles 1-4 with hydration and electrolyte support.	After the platinum phase, pembrolizumab + pemetrexed continue every 21 days when appropriate.	Immune toxicities can occur during treatment or after treatment stops.

IMPORTANT MEDICINE-SPECIFIC EFFECTS

Pembrolizumab - immune effects

- Immune inflammation: lungs, bowel, liver, hormone glands, kidneys, skin, heart, nerves or muscles.
- New immune symptoms can begin during treatment or after treatment has stopped.

Pemetrexed

- Low blood counts can increase infection, anaemia or bruising risk.
- Mouth/gut inflammation and skin rash can occur.
- Kidney function can deteriorate and is monitored.

Cisplatin

- Kidney injury and low magnesium/potassium can occur.
- Hearing loss or ringing in the ears may be permanent.
- Numbness/tingling and marked nausea/vomiting can occur.

OTHER / SHARED TREATMENT EFFECTS

The anatomy page highlights important symptom patterns for this exact regimen, but it cannot show every possible effect. Fatigue, nausea, appetite change, mouth/bowel symptoms or blood-count changes may occur depending on the medicines used. Report new or worsening symptoms even if they are not listed.

Monitoring / medicines

- Folic acid and vitamin B12 support are required with pemetrexed.
- Cisplatin requires hydration/electrolyte management and high-emetogenic-risk antiemetic support.
- Blood count, kidney/liver profile and glucose are checked; thyroid monitoring continues during pembrolizumab.

Bring / track

- Report immune symptoms early rather than waiting for the next cycle.

WHEN TO CALL - DO NOT WAIT FOR THE NEXT APPOINTMENT

Contact oncology urgently for fever, new breathlessness, persistent diarrhoea, jaundice, severe rash, chest pain, marked weakness, inability to maintain fluids, reduced urine or new neurological symptoms. For severe breathing difficulty, collapse, seizure or severe chest pain call 112/999.

Consent discussion support

Use the approved consent process to document treatment aim, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

<https://sactcheck.com/patient/00569/>

Official NCCP 00569 v5

SACTCheck v0.81.1 · patient information prototype for clinical review. Follow your oncology team and local emergency instructions.

